TEST TEST
MRN : **A1349836334**
Birthday : **1980-06-24**
Phone : **0000000000**
Visited on: 2024 Nov 19 13:06 (Age at visit: 44 years)
Electronically signed by: Andrea Pearson, MD, FAAD on 2024-11-19 01:29 PM

CC

Rash

Subjective

I am seeing this 44 year old, male patient from my home in Nelson, NH via 2 way syn. aud-vid comm and the patient is located in his home in DE.
Patient presents today for a tele-health visit for evaluation of a rash.
It began months to years. Constant during that time.
It is located on right lower leg and hip.
Patient describes dry, red, itchy patches
Quality: Itchy, not burning.
Associated symptoms: Otherwise well with no systemic symtpoms.
Prior evaluation/ treatment: Self. No prior visits. Vaseline. Humidifier. Minimal improvement.
New medications/ exposures: None.
Prior episodes/ history of rash: Dry skin but no diagnosis.
Skin care: Hot water, bar soap. Ivory or Dial.
Worse in the winter and better in the summer.
No history of food allergies, history of exercise induced asthma.

PMH: None
Medications: None
Allergies: None
FHx: Psoriasis in grandmother.

Occupation: Work in bicycle mechanics.
No travel.

Medications

No known medications

Allergies

No known allergies

Mental/Functional

He is well-appearing and in no acute distress, awake, alert and oriented x3 with appropriate mood and affect.

Vitals

Height/Length	Weight	BMI	Blood Pressure	Temperature	Pulse	RR	SpO2
6' 5"	**200 lbs 0 oz**	**23.72**					

Recorded: 2024 Nov 19 13:26 by: Andrea Pearson

Objective

General: well-appearing individual in no acute distress
HEENT: NCAT, extra-ocular movements intact, nares without discharge
Neck: appears supple, no hesitation with neck movement
CV: hands and fingers are pink and well-perfused
Lungs: comfortable respirations, no coughing, tachypnea, or retractions appreciated
Ext: moving upper extremities. well without hesitation
Skin: Skin unable to be visualized today.
Neuro: alert and oriented times three, normal speech, affect and behavior, facial cranial nerves are grossly intact.

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Assessment

Eczematous dermatitis; asteatotic eczema

Plan

Reviewed types of eczema and causes. Patient counseled on dry skin care:
Avoid hot water and long showers. Use Cetaphil cleanser rather than soap.
Moisturize with Cera Ve cream daily after showering.
Apply triamcinolone cream twice daily to affected areas and taper as improving.
Risks of steroid use and atrophy reviewed.
F/U in 6 weeks as needed or sooner if symptoms are not improving.
All questions answered to his satisfaction.

Greater than 20 minutes was spent in patient care, including review of prior records, and documentation. Greater than 50% of time was spent in direct patient care.